

Memorial Regional Hospital — Emergency Department

3501 Johnson St, Hollywood, FL 33021 | Level I Trauma Center
Phone: (954) 555-0600 | Fax: (954) 555-0601

Patient: Marta L. Jimenez

Date: 01/31/2026

DOB: 04/17/1958

Provider: Kevin Daly, MD

MRN: MRH-0882341

Specialty: Emergency Medicine

CHIEF COMPLAINT

Confusion and slurred speech — brought by daughter. Possible stroke per EMS.

HISTORY OF PRESENT ILLNESS

67-year-old Hispanic female brought by EMS after her daughter found her confused and speaking incoherently at home. Per daughter (primary historian — patient is minimally verbal and not providing reliable history): Patient was in her usual state of health yesterday morning. Daughter called her at 6 PM and she 'sounded drunk and confused.' Daughter arrived at the house at 8 PM and found her mother confused, slurring words, and not recognizing her. She had been incontinent of urine. No witnessed seizure activity. No facial droop per daughter, though she is not certain. No vomiting.

Patient has a history of cirrhosis (daughter believes from alcohol — patient 'used to drink a lot'), hypertension, and type 2 diabetes. She takes lactulose 'sometimes' per daughter but is not always compliant. She also takes lisinopril and metformin. Daughter reports patient has been constipated for 4-5 days. She has had two prior hospitalizations for 'liver problems' — daughter does not know details. She has not seen her doctor in over a year. Daughter is unaware of any recent infections, falls, or head trauma.

EMS documented: GCS 11 (E3V3M5), BP 162/96, glucose 78, no obvious facial asymmetry on field assessment. EMS stroke alert activated given altered mental status and slurred speech.

PHYSICAL EXAM

Vitals on arrival: BP 168/98, HR 102, Temp 37.8°C, RR 18, O2 sat 94% RA, Wt ~155 lbs

GCS: 12 (E3V4M5) — improved from field

General: Jaundiced female, mildly unkempt. Mild asterixis noted on outstretched hands. Spider angiomas on anterior chest. Palmar erythema. Moderate abdominal distension.

HEENT: Scleral icterus. Pupils 3mm, equal and sluggishly reactive. No obvious facial asymmetry at rest.

Cardiovascular: Tachycardic, regular. No murmurs.

Abdomen: Distended, dull to percussion in flanks — possible ascites. Mild diffuse tenderness without guarding.

Extremities: 2+ bilateral pitting edema to the knees. No focal limb weakness apparent.

Neuro: Confused, disoriented. Follows simple commands inconsistently. Speech slurred and slow. Cannot assess focal deficits reliably given cooperation. No obvious hemiplegia. Asterixis present bilaterally. Reflexes 2+ symmetric. Toes downgoing bilaterally.

ED COURSE AND WORKUP

Stroke team activated. CT head obtained (see report). IV access x2, O2 2L NC, monitor. Glucose 78 on finger stick — 1 amp D50 given empirically — minimal response. Labs sent (see lab report). Chest X-ray: cardiomegaly, no acute infiltrate. EKG: sinus tachycardia, no ischemic changes.

CT head returned as no acute hemorrhage but with chronic changes. Stroke team performed NIHSS: score 4 (confusion, mild dysarthria). tPA window considered — on hold pending labs and further evaluation. Neurology attending called to bedside.

ASSESSMENT AND PLAN

Assessment: Altered mental status with slurred speech in patient with cirrhosis. Initial concern for acute stroke given EMS activation, however clinical picture becoming more consistent with hepatic encephalopathy: asterixis, jaundice, stigmata of chronic liver disease, constipation, known cirrhosis, lactulose non-compliance, and fever. Stroke has NOT been definitively excluded — MRI with DWI would be needed but patient is confused and may not cooperate. tPA not administered given significant uncertainty about diagnosis and concern about coagulopathy in a cirrhotic patient.

1. Admit to medicine / hepatology
2. Lactulose 30 mL q2h until 3 soft BMs per day
3. Rifaximin 550 mg BID
4. IV thiamine 100 mg given prior to any dextrose
5. Blood cultures x2 for SBP workup given fever and ascites
6. Neurology consult ordered — please evaluate for stroke vs. metabolic encephalopathy
7. EEG ordered per neurology
8. MRI brain if patient's mental status and cooperation permit

Electronically signed by: Kevin Daly, MD

Signed 01/31/2026 23:58 EST | MRH Emergency EHR | Attending: Gloria Nwachukwu, MD

Memorial Regional Hospital — Department of Radiology

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RADIOLOGY REPORT

Patient: Marta L. Jimenez

DOB: 04/17/1958

MRN: MRH-0882341

Exam Date: 01/31/2026 22:10

Ordering MD: Kevin Daly, MD

Accession: MRH-R-20260131-9920

EXAM

CT HEAD WITHOUT CONTRAST — ACUTE STROKE PROTOCOL

CLINICAL INDICATION

67-year-old female with acute onset confusion and slurred speech. Stroke alert. Rule out hemorrhage.

TECHNIQUE

Non-contrast CT head. Standard stroke protocol with 5 mm axial slices and thin coronal/sagittal reformats.

COMPARISON

CT head from 03/2022 (outside study, available for comparison via PACS).

FINDINGS

Acute findings: No intracranial hemorrhage. No hyperdense vessel sign. No sulcal effacement or loss of gray-white differentiation to suggest early large territory infarction. ASPECTS score: 10/10.

Chronic/background findings:

- Multiple bilateral subcortical and periventricular hypodensities consistent with chronic small vessel ischemic disease (Fazekas grade 2-3). Findings are more prominent than on 2022 study, suggesting progression.
- A 6 mm hypodensity in the right posterior limb internal capsule and a 4 mm hypodensity in the left putamen are present — consistent with chronic lacunar infarcts, unchanged from prior study.
- Mild cortical volume loss and sulcal prominence consistent with age and possible alcohol-related atrophy.

Ventricles: Mildly prominent but stable compared to 2022. No hydrocephalus.

Posterior fossa: No acute cerebellar or brainstem lesion.

Calvarium: No fracture. No acute bony pathology.

Soft tissues: No scalp hematoma.

IMPRESSION

1. NO ACUTE INTRACRANIAL HEMORRHAGE.
2. No CT evidence of acute large territory infarction (ASPECTS 10). Acute ischemia cannot be excluded by CT alone — MRI with diffusion-weighted imaging is significantly more sensitive for acute infarct and is recommended.
3. Chronic small vessel ischemic disease, progressed since 2022.
4. Known chronic bilateral lacunar infarcts — right PLIC and left putamen — unchanged.
5. Mild cortical volume loss.

Clinical correlation required. In the appropriate clinical context, CTA head and neck should be considered for large vessel occlusion assessment if ischemic stroke remains suspected.

Electronically attested by: Mark Brennan, MD — Neuroradiology (Attending, on call)
Attested 01/31/2026 22:48 EST | MRH Radiology PACS

Memorial Regional Hospital — Clinical Laboratory

3501 Johnson St, Hollywood, FL 33021 | CLIA #10D0065831
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LABORATORY REPORT — COMPREHENSIVE PANEL

Patient: Marta L. Jimenez DOB: 04/17/1958 MRN: MRH-0882341
Exam Date: 01/31/2026 22:15 Ordering MD: Kevin Daly, MD Accession: LAB-20260131-6641

COLLECTION: 01/31/2026 22:15 | REPORTED: 02/01/2026 00:45

BASIC METABOLIC PANEL

Test	Result	Reference	Flag
Sodium	128	136-145 mEq/L	H
Potassium	3.2	3.5-5.0 mEq/L	L
Chloride	94	98-107 mEq/L	L
CO2	20	22-29 mEq/L	L
BUN	38	7-20 mg/dL	H
Creatinine	1.4	0.6-1.1 mg/dL	H
eGFR (CKD-EPI)	38	≥60 mL/min/1.73m²	L
Glucose	82	70-99 mg/dL	

LIVER FUNCTION TESTS

Test	Result	Reference	Flag
AST (SGOT)	188	10-40 U/L	H
ALT (SGPT)	72	7-56 U/L	H
Total Bilirubin	8.4	0.2-1.2 mg/dL	H
Direct Bilirubin	5.1	0-0.3 mg/dL	H
Alkaline Phosphatase	214	44-147 U/L	H
GGT	310	9-48 U/L	H
Total Protein	5.8	6.3-8.2 g/dL	L
Albumin	2.4	3.5-5.0 g/dL	L
Prothrombin Time (PT)	18.2 sec	11.0-14.0 sec	H
INR	1.62	0.8-1.2	H

AMMONIA & SPECIAL

Test	Result	Reference	Flag
Ammonia (plasma)	142 µmol/L	11-48 µmol/L	H
Lactate	3.1 mmol/L	0.5-2.2 mmol/L	H
Lipase	48	13-60 U/L	
TSH	2.8	0.4-4.0 mIU/L	
Vitamin B12	312	200-900 pg/mL	
Blood Culture x2	Pending 5 days	Negative	

CBC

Test	Result	Reference	Flag
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WBC	11.4	4.5-11.0 K/ μ L	H
Hemoglobin	9.8	12.0-16.0 g/dL	L
MCV	102	80-100 fL	H
Platelets	68	150-400 K/ μ L	L
Neutrophils %	78	40-75%	H

TOXICOLOGY

Test	Result	Reference	Flag
Serum Ethanol	<10 mg/dL (undetectable)	<10 mg/dL	
Urine Drug Screen	Negative (all panels)	Negative	
Salicylate level	<1.0 mg/dL	<20 mg/dL	
Acetaminophen level	<10 μ g/mL	<20 μ g/mL	

CRITICAL VALUES CALLED: Sodium 128 mEq/L (critical low) called to RN at 00:52. Ammonia 142 μ mol/L (critical high) called to RN at 00:52. INR 1.62 flagged. Platelets 68K — thrombocytopenia.

SUMMARY NOTE: Findings consistent with decompensated liver cirrhosis: markedly elevated ammonia, conjugated hyperbilirubinemia, hypoalbuminemia, coagulopathy (elevated PT/INR), thrombocytopenia, and hyponatremia. Elevated lactate may reflect hepatic dysfunction and/or early sepsis. Macrocytic anemia (MCV 102) consistent with hepatic disease or prior alcohol use. Ethanol undetectable at time of draw.

Memorial Regional Hospital — Neurophysiology Laboratory

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ELECTROENCEPHALOGRAPHY (EEG) REPORT — PORTABLE BEDSIDE

Patient: Marta L. Jimenez	DOB: 04/17/1958	MRN: MRH-0882341
Exam Date: 02/01/2026 06:30	Ordering MD: Raymond Osei, MD (Neurology)	Accession: MRH-EEG-20260201-0114

CLINICAL INDICATION

67-year-old female admitted for altered mental status. Known cirrhosis. Ammonia elevated (142 $\mu\text{mol/L}$). Evaluate for nonconvulsive seizures vs. encephalopathy pattern.

RECORDING DETAILS

Portable bedside EEG performed in medical ICU. 21-electrode standard 10-20 placement. Recording duration: 30 minutes. Patient encephalopathic — drowsy to lethargic throughout. No hyperventilation performed (patient unable to cooperate). Photoc stimulation not performed.

BACKGROUND ACTIVITY

No normal posterior dominant rhythm identified. Background consists predominantly of moderate-amplitude diffuse theta (4-7 Hz) and delta (0.5-3 Hz) activity. No normal sleep architecture. No normal waking background for age. The background is diffusely slow and disorganized — no regional predominance.

INTERICTAL / ABNORMAL FINDINGS

Prominent generalized, bilaterally synchronous triphasic waves (TWs) are identified throughout the recording. These are high-amplitude, frontally predominant, with a characteristic morphology: positive-negative-positive complex with anterior-to-posterior phase lag. Frequency approximately 1.5-2.5 Hz. They persist throughout all states of arousal captured. No clear focal onset epileptiform discharges identified. No electrographic seizure activity recorded.

Attempted stimulation of the patient during recording produced brief attenuation of the pattern followed by resumption — consistent with encephalopathy, not seizure.

REACTIVITY

EEG mildly reactive to stimulation with brief attenuation. No normal reactivity pattern.

NO SEIZURE ACTIVITY

No electrographic or clinical seizures recorded during this 30-minute study. A longer recording or continuous EEG monitoring may be considered if nonconvulsive status epilepticus (NCSE) remains a clinical concern.

IMPRESSION

1. ABNORMAL EEG — moderately-severely abnormal.
2. Diffuse background slowing with prominent generalized triphasic waves. This pattern is classically associated with **metabolic encephalopathy**, most commonly hepatic encephalopathy (in this clinical context), uremic encephalopathy, or other toxic-metabolic derangements. The pattern is not specific to etiology but is highly consistent with the patient's clinical profile and markedly elevated ammonia.
3. No epileptiform discharges. No electrographic seizures in this recording.
4. Triphasic waves can occasionally be difficult to distinguish from NCSE; if clinical suspicion for NCSE remains, a benzodiazepine trial under EEG monitoring or continuous EEG can be considered. However, the pattern here is typical of metabolic encephalopathy rather than NCSE.

Recommend clinical correlation with ammonia, liver function, and metabolic parameters. EEG findings support a primary metabolic/toxic etiology over structural or epileptic cause.

